جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : فاطمه حسين محمود احمد
المحافظة : القاهره **الحي / المدينة :** المطريه **العنوان :** ~~
رقم البطاقة : 25807091400401 **رقم التليفون :** 01157134024 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 09-07-1958 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة الإجابات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فاطمه حسين محمود احمد
National ID: 25807091400401

GUSTAVE ROUSSY

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : ---:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Name : Mrs Fatma Hussien Mahmoud
Date : 5 April 2026

ECHOCARDIOGRAPHY REPORT

LVEDD	56	38-57 mm	Ao Root	27	<40 mm
LVESD	42	22-40 mm	LA:	41	<40mm
IVS	9	7-11 mm	ACS	16	10-20 mm
PWT	6	7-11 mm	EF:	49%	55-77%

Comments:

- ***Increased left ventricular end-systolic internal dimensions with mildly impaired systolic function. Estimated EF 49% by M mode & by 2D eyeballing.***
- ***Akinetic, thinned basal and mid inferior& infero-lateral wall segments***
- *LV diastolic dysfunction grade II.*
- *Dilated left atrial diameter with normal aortic root diameter.*
- *Normal mitral valve leaflets with mild MR.*
- *Sclerotic AV morphology with normal flow.*
- ***Normal TV morphology with modertae TR & estimated RVSP=50 mmHg.***
- *Dilated Right side with preserved RV systolic function.*
- *No pericardial effusion.*
- ***Pacemaker wire clearly seen in the Rt side.***

Best Regards,
Dr. Ibrahim Sabry
Egyptian fellowship of cardiology


جمهورية مصر العربية
بطاقة تحقيق الشخصية
فاطمه
حسون محمود احمد العرقسوسى
١٢ ش محمد متطاوى مسطرد
شبرا الخيمه ثان - القليوبية
٢٥٨٠٧٠٩١٤٠٠٤٠١
JL5453230


٢٠٢٢/٠٦
٢٥٨٠٧٠٩١٤٠٠٤٠١
ربه منزل
انثى
مسلمة
متزوجة
عبدالعال سعد عبدالعال محمد
البطاقة سارية حتى ٢٠٢٩/٠٦/٢٨
١١

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

03/18/2026

MDT FORM

Name:	فاطمه حسين محمود احمد	**MRN:**	8679
Age:	68 Y	**ID:**	25807091400401
Menopausal Sta	POST	**Phone:**	01270413066
NRS SCORE:	1	**Marital Status:**	MARRIED
BMI:	33	**Family History:**	-VE
CO Morbidity:	DM,..PACEMAKER,,CARDIAC STENTS	**Reason Of MDT:**	NEW CASE

Clinical Data:

BX: IDC GII

ER: 5/8 **PR:** 5/8 **HER2:** +3 **KI67:** 25

Sonomammography:
LT BREAST UOQ 43*24MM 6 CM FROM NIPPLE, MULTIPLE AXILLARY LNS AND SUPRA CLAVECULAR
RT BREAST PROMINANT DUCTS

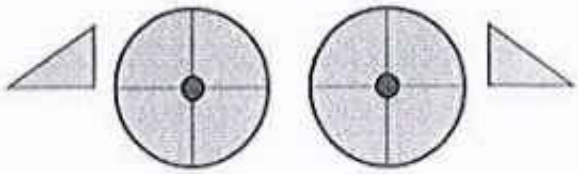
Mets Workup:
CT CAP,,, EXCEPT FOR MEDIASTINAL SUSPICIOUS LNS AND LT BASAL PULMONARY NODULE FOR FOLLOW UP

History:

Recommendations:
FOR NACT AND POSSIPLE MRM ACCORDING TO RESPONSE

MDT Outcome & Plan
after CT reviewing by radiodiagnosis: metastatic mediastinal LNs but no other sites.
for Echo - Port cath insertion then,
to start systemic treatment Herceptin + weekly Taxol 4 months and reassess

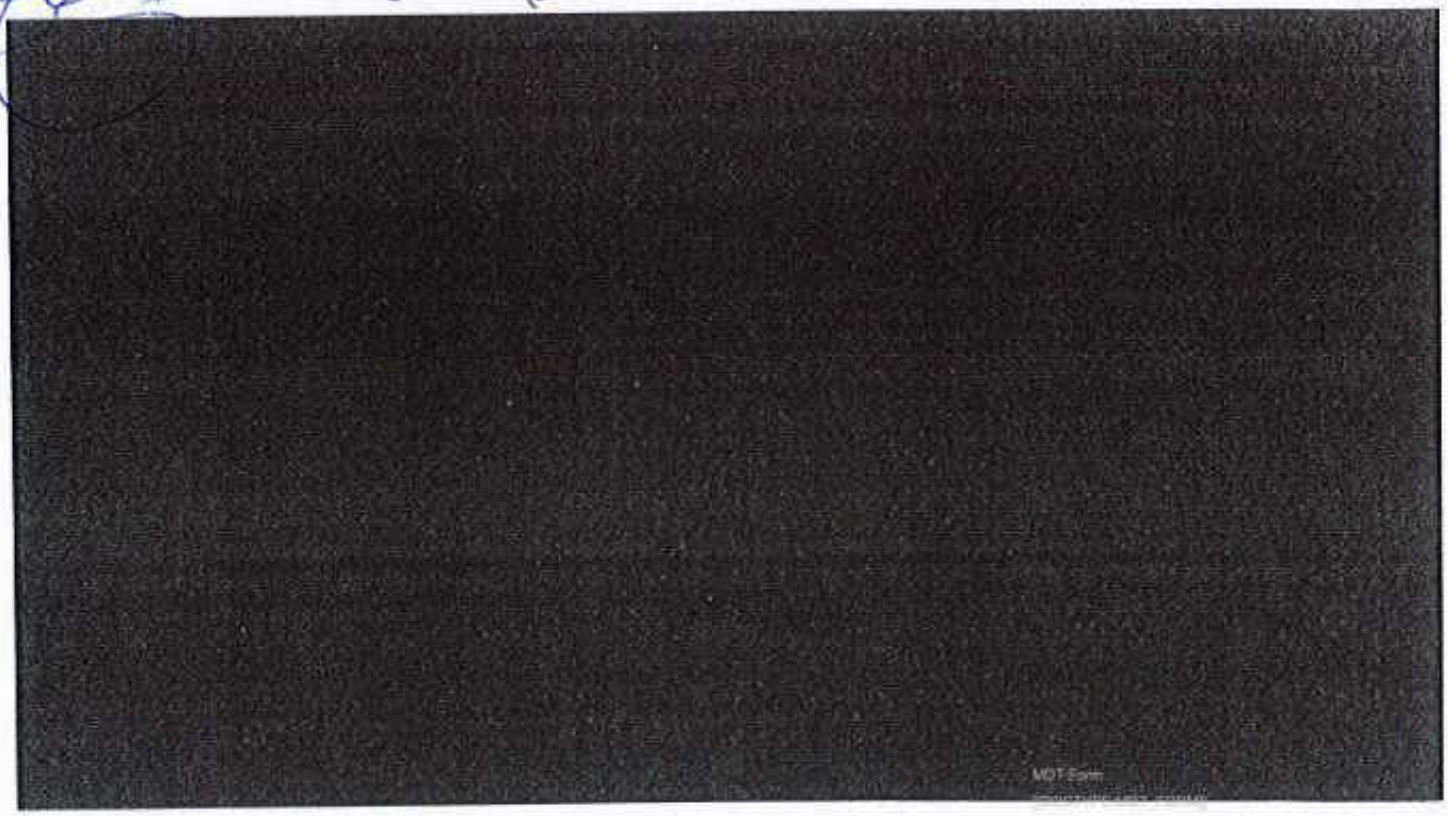
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TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نادى حسن	د/ اسامه عبدوى	د/ ترجمان رمضان			



GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / فاطمه حسين محمود	Request No.	: 13005031
Patient No.	: 25807091400401	Request Date	: 14/02/2026
Path No.	: SR-242/26	Verification Date	: 19/02/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 67 Y	Department	: Out Patient
Referreing Doc	: Dr. Mohamed El Nabarawy□	Registered by	: Mrs.Mayada Hegazy

Pathology Report

- PR: Positive (score 5/8).
- Her2: Positive (score 3+).
- Ki67%: 25%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low) membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
DOCTYPE PATHOLOGY REPORTS

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / فاطمه حسين محمود	Request No.	: 13005031
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Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 67 Y	Department	: Out Patient
Referreing Doc	: Dr. Mohamed El Nabarawy□	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast UOQ mass.
Nature of Specimen: Referred block & slide (Tru-cut biopsy).

Gross Examination:
- One referred block & one referred (H&E) stained slide were received, labelled by (1029376).
Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Angioinvasion: Present.
* Perineural invasion: Absent.
* IPT (Positive control showed appropriate reactivity):
- ER: Moderate nuclear reaction in 20% of tumor cells (score 5/8).
- PR: Strong nuclear reaction in 10% of tumor cells (score 5/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 25%.

Diagnosis:
Left breast mass, Referred block & slide (Tru-cut biopsy):
- Invasive duct carcinoma, Grade 2.
IPT:
- ER: Positive (score 5/8).

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

AL-Sahel Teaching Hospital
Radiodiagnosis Department

مستشفى الساحل التعليمي
قسم الأشعة التشخيصية

Patient Name: FATMA HUSSIN MAHMOUD	Age: 67 Years
Patient ID: 25803091400401	Study Date: 21/01/2026 12:23:32

Bilateral Breasts Sonomammography Examination

Mammography revealed:

- ACR :C heterogenous glandular pattern
- Left breast shows UOQ asymmetrical density with grouped microcalcifications associated with mild increase density, trabeculations and skin thickness.
- Left pace maker is seen.

The complementary ultrasound shows:

- Both breasts show fibroglandular tissue.
- The left breast shows an ill defined irregular hypoechoic lesion measuring 43.7x24.3mm seen in the UOQ at 2 oclock 6cm from the nipple with internal vascularity. Ipsilateral multiple enlarged axillary pathological LNs, the largest measuring 16x10mm with increased cortical thickness= 6mm. Associated supraclavicular LN measuring 10x7mm.
- The right breast shows retroareolar prominent duct 2.7mm with echogenic intraductal content.
- No left dilated ducts seen.
- Normal nipple areolar complex.
- Right axillary non specific lymph nodes with preserved hilum.

Conclusion:

Left breast UOQ highly suspicious lesion with grouped microcalcifications and ipsilateral multiple enlarged axillary pathological LNs and supraclavicular LN as described (BIADSV) for further biopsy and histopathological assessment.
Right retroareolar prominent duct with intraductal contents.

Best Regards,
Dr. Iriny Salib

تحويل للأشعة لأخذ العينة

د/ [illegible]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

٨٦٧٩

Name : Fatema Hussain Mahmoud	**Date : 02-03-2026 02:21**
Branch : Portable	**Medical No. : GRI-25807091400401**
Ref.Physician Prof : CAP+C	**Ins.Company :**

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Findings:

- Left breast outer quadrant soft tissue mass measures 3.7x2.8 cm.
- Left axillay pathological looking lymphadenopathy 14 mm.
- Mediastinal suspicious lymphadenopathy at prevascular group measures 20 mm.
- Left basal subpleural pulmonary nodule measures 4.4 mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Cardiomegaly. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

OPINION:

- Left breast outer quadrant soft tissue mass for US correlation.
- Left axillary pathological looking lymphadenopathy.
- Mediastinal suspicious lymphadenopathy.
- Left basal pulmonary nodule for follow up.

CT Scan
[DOCTYPE:CT_SCAN]

GUSTAVE ROUSSY
INTERNATIONAL CENTER

Name : Fatema Hussain Mahmoud
Branch : Portable
Ref.Physician Prof : CAP+C

Date : 02-03-2026 02:21
Medical No. : GRI-25807091400401
Ins.Company :

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH ORAL & I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

- ***Normal CT scan of the abdomen and pelvis.***

Dr Mai Mohamed MD

CT Scan
[DOCTYPE:CT_SCAN]

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : فاطمه حسين محمود احمد العرفسوسى Analysis ID : 1029376

Age : 67 Y : 6 M : 1 D Gender : Female Conf. Code : 6358

Referred By Prof. Dr. : السعيد احمد

Clinical Diagnosis : Left breast mass UOQ, malignant looking mas, likely attached to chest wall muscle, with pathological left axillary and left supraclavicular lymph nodes.

Nature of Specimen : US guided tru cut biopsy.

Receiving Date : 2026-01-28 Patient ID : 756694

PATHOLOGY REPORT

Gross :

Multiple greyish white tissue cores, the longest measured 1 cm long, totally submitted.

Microscopic :

- Serial sections examined from the specimen received revealed cores of tumour tissue formed of malignant ductal epithelial cells, showing moderate to marked nuclear pleomorphism and hyperchromasia. There is poor attempt at tubular formation.
There are moderate number of mitotic figures.
There is marked desmoplasia.
There is peritumoural lymphocytic infiltrate around 3 %.
There is focal necrosis.
There is intraductal component about less than 5 % showing cribriform pattern.

Diagnosis :

Left breast mass, US guided tru cut biopsy
, INFILTRATING DUCT CARCINOMA, GRADE III.

Prof. Dr. Shady Elia Anis *Prof. Dr. Elia Anis Ishak*

This report is electronically signed , any abrasion or modification would render it void .

DR ELIA ANIS LABS
عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧

GUSTAVE ROUSSY
INTERNATIONAL

جوستاف روسي

ECHO REPORT

NAME : فاطمة حسن DATE: 29/9/202

ID NUMBER : / SEX : AGE:

EF	55%	55-75%	PWT	1.0	0.6-1.1cm
LVEDD	5.4	3.5-5.8cm	AO.R		3-3.7cm
LVESD	3.1	2-4cm	LA	4.0	2-4cm
FS		25-45%	RV		0.7-2.3com(supine)
SWT	1.2	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions
LV Wall Thickness
LV Diastolic Function Dysn
SWMA DOT 55%
LA and Ao Root Dimensions Dilated A.
Mitral Valve mild MR
Aortic Valve
Tricuspid Valve
Pulmonary Valve
Right Side and Systolic Function
Intracardiac Masses and Effusion

IHD
SLE 4 stents

DR:

Patient Name: Mrs. Fatma Hussein Mahmoud Ahmed	Pat. ID: 751262
Study Date: 27/06/2026	Ref. Doctor: Em Em
Medical File Num.: 0	

CT SCAN OF THE CHEST, ABDOMEN AND PELVIS AFTER ORAL AND I.V CONTRAST ADMINISTRATION REVEALED:-

A Know patient with breast cancer received CTH

I- CHEST:

- Right porta cath is noted.
- Cardiac pace maker is noted.
- Right lower lobar sub segmental atelecatasis is seen.
- Diffuse patchy pulmonary veiling is seen.
- Right lower lobar posterior sub pleural reticulations are seen.
- No pulmonary nodules or cavitary lesions.
- Multiple enlarged hilar, mediastainal and cardio-phrenic lymph nodes are seen, the largest measures 1.3x0.7 cm at pre vascular region.
- No pleural or pericardial collection.
- Cardiomegaly is seen with average diameters of thoracic aorta and main pulmonary trunk.
- Enlarged left axillary lymph nodes are seen.

II-ABDOMEN AND PELVIS:

- Enlarged liver showing few rather defined right lobe hypo dense focal lesions with no specific pattern of enhancement, the largest is seen at hepatic dome and roughly measuring 4x1.8 cm.
- Patent homogenously enhancing portal vein and its main branches.
- No biliary dilatation.
- Normal C.T appearance of the spleen, adrenal glands, pancreas, aorta and IVC.
- Average sized, normally located both kidneys are seen with preserved renal parenchyma.

- Left ureteric upper and lower segmental circumferential mural thickening and enhancement is seen with mild proximal segmental dilatation and dilated left renal pelvis.
- Smudged left perinephric fat planes are seen.
- Averagely distended urinary bladder is see with mild bladder dome mural thickening and strand supra vesical fat plaes yuet no gross masses.
- No pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- L1 vertebral body mild anterior wedging is seen with no associated retropulsion or soft tissue components.

Opinion:

- **Right lower lobe sub segmental atelectasis.**
- **Enlarged mediastinal lymph nodes.**
- **Few right hepatic lobe focal lesions likely metastatic.**
- **Left ureteric segmental circumferential mural thickening and enhancement as well as bladder dome mural thickening ? inflammatory.**
- **L1 vertebral body mild anterior wedging.**

Much Obliged;

Dr. Ahmed Adel, FEBR

Electronically Signed By: ahmed adel

06/28/2026 02:43 م

Patient Name: Mrs. Fatma Hussein Mehmoud Ahmed	Pat. ID: 751262
Study Date: 27/08/2026	Ref. Doctor: Em Em
Medical File Num.: 0	

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- **Enlarged mediastinal lymph nodes.**
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- **Left ureteric segmental circumferential mural thickening and enhancement as well as bladder dome mural thickening ? inflammatory.**
- **L1 vertebral body mild anterior wedging.**

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Dr. Ahmed Adel, FEBR

Electronically Signed By: ahmed adel

06/28/2026 02:43 م

Patient Name: Mrs. Fatma Hussein Mahmoud Ahmed	Pat. ID: 751262
Study Date: 27/08/2026	Ref. Doctor: Em Em
Medical File Num.: 0	

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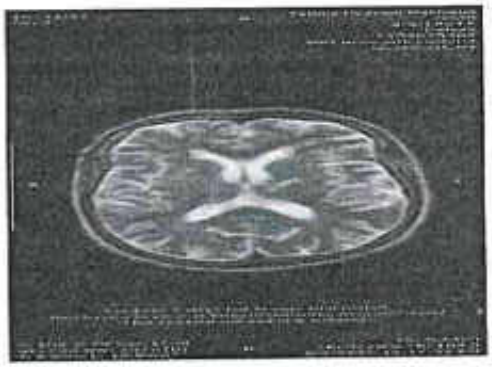
Much Obliged;

Dr. Ahmed Adel, FEBR

Electronically Signed By: ahmed adel

06/28/2026 02:43

Patient's Name:	**Fatima Hussein Mahmoud**
Date:	**2026-08-03**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**



MRI EXAMINATION OF THE BRAIN

MR Technique:

- Axial T1, T2 and FLAIR WI's
- Sagittal T1WI
- Coronal T2 WI
- Axial DWI and ADC
- Axial,sagittal and coronal T1WIs with contrast

MR findings:

- Mildly increased peri-ventricular deep white matter signal,denoting ischemia.
- Tiny foci of alyered signal are seen in both lentiform nuclei,posterior limb of left internal capsule,left frontal and both parietal lobes. No evidences of enhancement or foci of restricted diffusion.
- Prominance of CSF-containing structures.
- No other cerebral, cerebellar or brain stem areas of altered MR signal/enhancement/blood signal/or restricted diffusion on DW and ADC sequences.
- No midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.
- Normal sellar and para-sellar regions.
- Intact cervico medullary junction.
- **Cuts taken in the orbits are unremarkable.**
- **Cuts taken in the mastoid air cells revealed right mastoiditis.**
- **Cuts taken in the para-nasal sinuses revealed mild ethmoidal sinusitis with a tiny left maxillary polyp measuring about 7x1.4 mm in dimensions.**

Impressions:

- **Brain involutional changes.**

جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السفير المصرية صحة مصر

06/29/2026

MDT FORM

Name:	فاطمه حسين محمود احمد	**MRN:**	8679
Age:	68 Y	**ID:**	25807091400401
Menopausal Sta	post	**Phone:**	01286206555
NRS SCORE:	ps 1	**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:	dilated cardiomyopathy ef 45%	**Reason Of MDT**	

Clinical Data:

BX:

ER: **PR:** **HER2:** **KI67:**

Sonomammography:

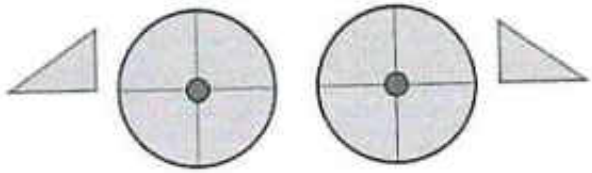
Mets Workup:

History:
wt 79 ht 150 sa 1.8 postm left breast cancer ct3 n3 m1 med in idc 2 er5/8 pr5/8 her2 3+ cts med in bs free ef 49
pt rec 8 weeks taxol only
cts done on 27/6/2026; med in 1.3x0.7 newly developped rt hfl largest in hepatic dome m 4x1.8 for mdt (hfl was not present in initial cts)

Recommendations:

MDT Outcome & Plan
for 1. cardiology consultation
2. CT comparison
3. if contraindicated for herceptin, for tykerb + xeloda

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د شهاب الاشين	داليه عيسوي				

20/7/2026 20/7/2026

Patient Name: Mrs. Fatma Hussein Mehmoud Ahmed	Pat. ID: 751262
Study Date: 27/06/2026	Ref. Doctor: Em Em
Medical File Num.: 0	

CT SCAN OF THE CHEST, ABDOMEN AND PELVIS AFTER ORAL AND I.V CONTRAST ADMINISTRATION REVEALED:-

A Know patient with breast cancer received CTH

I- CHEST:

- Right porta cath is noted.
- Cardiac pace maker is noted.
- Right lower lobar sub segmental atelecatasis is seen.
- Diffuse patchy pulmonary veiling is seen.
- Right lower lobar posterior sub pleural reticulations are seen.
- No pulmonary nodules or cavitary lesions.
- Multiple enlarged hilar, mediastainal and cardio-phrenic lymph nodes are seen, the largest measures 1.3x0.7 cm at pre vascular region.
- No pleural or pericardial collection.
- Cardiomegaly is seen with average diameters of thoracic aorta and main pulmonary trunk.
- Enlarged left axillary lymph nodes are seen.

II-ABDOMEN AND PELVIS:

- Enlarged liver showing few rather defined right lobe hypo dense focal lesions with no specific pattern of enhancement, the largest is seen at hepatic dome and roughly measuring 4x1.8 cm.
- Patent homogenously enhancing portal vein and its main branches.
- No biliary dilatation.
- Normal C.T appearance of the spleen, adrenal glands, pancreas, aorta and IVC.
- Average sized, normally located both kidneys are seen with preserved renal parenchyma.

- Left ureteric upper and lower segmental circumferential mural thickening and enhancement is seen with mild proximal segmental dilatation and dilated left renal pelvis.
- Smudged left perinephric fat planes are seen.
- Averagely distended urinary bladder is see with mild bladder dome mural thickening and strand supra vesical fat plaes yuet no gross masses.
- No pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- L1 vertebral body mild anterior wedging is seen with no associated retropulsion or soft tissue components.

Opinion:

- **Right lower lobe sub segmentall atelectasis.**
- **Enlarged mediastinal lymph nodes.**
- **Few right hepatic lobe focal lesions likely metastatic.**
- **Left ureteric segmental circumferential mural thickening and enhancement as well as bladder dome mural thickening ? inflammatory.**
- **L1 vertebral body mild anterior wedging.**

Much Obliged;

Dr. Ahmed Adel, FEBR

Electronically Signed By: ahmed adel

06/28/2026 02:43 م

GUSTAVE ROUSSY
INTERNATIONAL

٨٦٧٩

Name : Fatema Hussain Mahmoud	**Date : 02-03-2026 02:21**
Branch : Portable	**Medical No. : GRI-25807091400401**
Ref.Physician Prof : CAP+C	**Ins.Company :**

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Findings:

- Left breast outer quadrant soft tissue mass measures 3.7x2.8 cm.
- Left axillay pathological looking lymphadenopathy 14 mm.
- Mediastinal suspicious lymphadenopathy at prevascular group measures 20 mm.
- Left basal subpleural pulmonary nodule measures 4.4 mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Cardiomegaly. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

OPINION:

- Left breast outer quadrant soft tissue mass for US correlation.
- Left axillary pathological looking lymphadenopathy.
- Mediastinal suspicious lymphadenopathy.
- Left basal pulmonary nodule for follow up.

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

Name : Fatema Hussain Mahmoud
Branch : Portable
Ref.Physician Prof : CAP+C

Date : 02-03-2026 02:21
Medical No. : GRI-25807091400401
Ins.Company :

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH ORAL & I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

- ***Normal CT scan of the abdomen and pelvis.***

Dr Mai Mohamed MD

CT Scan
(DOCTYPE:CT_SCAN)

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / فاطمه حسين محمود	Request No.	: 13005031
Patient No.	: 25807091400401	Request Date	: 14/02/2026
Path No.	: SR-242/26	Verification Date	: 19/02/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 67 Y	Department	: Out Patient
Referreing Doc	: Dr. Mohamed El Nabarawy□	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast UOQ mass.
Nature of Specimen: Referred block & slide (Tru-cut biopsy).

Gross Examination:
- One referred block & one referred (H&E) stained slide were received, labelled by (1029376).
Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Angioinvasion: Present.
* Perineural invasion: Absent.
* IPT (Positive control showed appropriate reactivity):
- ER: Moderate nuclear reaction in 20% of tumor cells (score 5/8).
- PR: Strong nuclear reaction in 10% of tumor cells (score 5/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 25%.

Diagnosis:
Left breast mass, Referred block & slide (Tru-cut biopsy):
- Invasive duct carcinoma, Grade 2.
IPT:
- ER: Positive (score 5/8).

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / فاطمه حسين محمود	Request No.	: 13005031
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Age	: 67 Y	Department	: Out Patient
Referreing Doc	: Dr. Mohamed El Nabarawy□	Registered by	: Mrs.Mayada Hegazy

Pathology Report

- PR: Positive (score 5/8).
- Her2: Positive (score 3+).
- Ki67%: 25%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low) membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

جمهورية مصر العربية
بطاقة تحقيق الشخصية



فاطمه
حسين محمود احمد العرقسوسى
١٢ ش محمد منطاوى .مسطرد
شبرا الخيمه ثان - القليوبية

٢٥٨٠٧٠٩١٤٠٠٤٠١

JL5453230

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د/ عبدالعال سعد
٢٠٢٢/٩

٢٥٨٠٧٠٩١٤٠٠٤٠١ ٢٠٢٢/٠٩
ربه منزل

انثى مسلمة متزوجة
عبدالعال سعد عبدالعال محمد
١١ البطاقة سارية حتى ٢٠٢٩/٠٦/٢٨



National ID - Front & Back
(IDCOTYPE.ID_FRONT_BACK)

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : فاطمه حسين محمود احمد العرقسوسى **Analysis ID :** 1029376

Age : 67 Y : 6 M : 1 D **Gender :** Female **Conf. Code :** 6358 ٨٧٧٩

Referred By Prof. Dr. : السعيد احمد

Clinical Diagnosis : Left breast mass UOQ, malignant looking mas, likely attached to chest wall muscle, with pathological left axillary and left supraclavicular lymph nodes.

Nature of Specimen : US guided tru cut biopsy.

Receiving Date : 2026-01-28 **Patient ID :** 756694

PATHOLOGY REPORT

Gross :

Multiple greyish white tissue cores, the longest measured 1 cm long, totally submitted.

Microscopic :

- Serial sections examined from the specimen received revealed cores of tumour tissue formed of malignant ductal epithelial cells, showing moderate to marked nuclear pleomorphism and hyperchromasia. There is poor attempt at tubular formation.
There are moderate number of mitotic figures.
There is marked desmoplasia.
There is peritumoural lymphocytic infiltrate around 3 %.
There is focal necrosis.
There is intraductal component about less than 5 % showing cribriform pattern.

Diagnosis :

Left breast mass, US guided tru cut biopsy
, INFILTRATING DUCT CARCINOMA, GRADE III.

Prof. Dr. Shady Elia Anis ***Prof. Dr. Elia Anis Ishak***

This report is electronically signed , any abrasion or modification would render it void .

DR ELIA ANIS LABS
عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧

Dr.
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M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : فاطمه حسين محمود احمد العرقسوسى **Analysis ID :** 1031423

Age : 67 Y : 6 M : 2 D **Gender :** Female **Conf. Code :** 5325

Referred By Prof. Dr. : السعيد احمد

Clinical Diagnosis : Left breast mass UOQ, malignant looking mas, likely attached to chest wall muscle, with pathological left axillary and left supraclavicular lymph nodes.

Nature of Specimen : Block (Labelled as 1029376)

Receiving Date : 2026-02-04 **Patient ID :** 756694

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain .
The sections were treated against:
- Estrogen receptors -Progesterone receptors - Her 2 - Ki67

Results :

15% of tumor cells show STRONGLY POSITIVE Staining for Estrogen receptors
10% of tumor cells show STRONGLY POSITIVE Staining for progesterone receptors.
Sections treated for HER2 revealed STRONG CONTINUOUS CELL MEMBRANE STAINING (CHICKEN WIRE) IN MORE THAN 10%OF TUMOR CELLS.
25% of Tumour cells are POSITIVE for Ki67.

Conclusion :

Block, TUMOUR CELLS ARE
POSITIVE FOR ESTROGEN RECEPTORS (SCORE 6 / 8)
POSITIVE FOR PROGESTERONE RECEPTORS (SCORE 5/ 8)
HER 2 POSITIVE, SCORE 3+
THE PROLIFERATION INDEX OF Ki67 IS 25%.

Prof. Dr. Shady Elia Anis ***Prof. Dr. Elia Anis Ishak***

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DR ELIA ANIS LABS
عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧

AL-Sahel Teaching Hospital
Radiodiagnosis Department

مستشفى الساحل التعليمي
قسم الأشعة التشخيصية

Patient Name: FATMA HUSSIN MAHMOUD	Age: 67 Years
Patient ID: 25803091400401	Study Date: 21/01/2026 12:23:32

Bilateral Breasts Sonomammography Examination

Mammography revealed:

- *ACR :C heterogenous glandular pattern*
- *Left breast shows UOQ asymmetrical density with grouped microcalcifications associated with mild increase density , trabeculations and skin thickness.*
- *Left pace maker is seen.*

The complementary ultrasound shows:

- ***Both breasts*** *show fibroglandular tissue.*
- ***The left breast shows*** *an ill defined irregular hypoechoic lesion measuring 43.7x24.3mm seen in the UOQ at 2 oclock 6cm from the nipple with internal vascularity. Ipsilateral multiple enlarged axillary pathological LNs, the largest measuring 16x10mm with increased cortical thickness= 6mm. Associated supraclavicular LN measuring 10x7mm.*
- ***The right breast shows*** *retroareolar prominent duct 2.7mm with echogenic intraductal content.*
- *No left dilated ducts seen.*
- *Normal nipple areolar complex.*
- *Right axillary non specific lymph nodes, with preserved hilum.*

Conclusion:

Left breast UOQ highly suspicious lesion with grouped microcalcifications and ipsilateral multiple enlarged axillary pathological LNs and supraclavicular LN as described (BIADSV) for further biopsy and histopathological assessment.
Right retroareolar prominent duct with intraductal contents .

Best Regards,
Dr. Iriny Salib

تحويل للأشعة لأخذ العينة

دا السيد عمر

Nuclear Medicine Unit وحدة الطب النووى

Fatma Hussien Mahmoud *Date: 09/02/2026*

Tc99m MDP SKELETAL SCINTIGRAPHY

History

- A68-year-old femalewithhistory ofbreast cancer.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- No definite evidence of bone secondaries could be seen.
- Features suggestive of degenerative changes of the spine and big joints.

IMPRESSION

- The skeleton is scintigraphically free from metastases.

Appreciated,

Dr.Hossam Sekkina, M.Sc.

Dr. Alaa Nagi, MD
Nuclear Medicine

الهيئة العامة
للمستشفيات والمعاهد التعليمية
مستشفى المطرية التعليمي
قسم الاشعة

Name | Fatma Hussein Mahmoud
Date | 5/1/2026

◘ Pelvi-abdominal U/S reveals:

- **Liver:** is average in size (15.5in MCL) with homogenous parenchymal echopattern. No focal lesions or intra-hepatic biliary radicle dilatation.
- **PV & CBD**. (both are not dilated).
- **Gall bladder:** is average in size with normal wall thickness. No stones or mud.
- **Both kidneys:** are within average size and normal site and shape. Normal parenchymal thickness with preserved cortico-medullary differentiation. No stones, cysts or back pressure changes on either side.
- **Spleen:** is average in size (8 cm along its longitudinal axis) with homogenous texture and no focal lesions. Splenic vein is not dilated.
- **Pancreas and para-aortic area:** no gross lesion could be detected sonographically.
- **Urinary bladder:** sub optimally distension with normal wall thickness. No stones, masses or diverticular out pouching.
- No gross pelvic masses or cysts.
- Colonic gaseous distention hindering proper pelvic examination.

◘ Conclusion:
Unremarkable study

Much obliged,

Dr. Esraa Salah